

आक्षेपक (Ākṣepaka)

पाणिपादशिरःपृष्ठश्रोणीः स्तम्भाति मारुतः ॥
दण्डवत्तद्व्याघ्रात्रस्य दण्डकः सोऽनुपक्रमः ।
C.S.Ci. 28/50-51

A convulsion is a medical condition where body muscles contract and relax rapidly and repeatedly, resulting in an uncontrolled shaking of the body. A type of paroxysmal and spasmodic involuntary movements is considered in this category. Generally, convulsion is a symptom, which is observed in various clinical conditions. Excessively aggravated *vāta* causes frequent convulsions in different parts of the body drying up of the hands & legs along with blood vessels, ligaments & tendons. It is known as *Ākṣepaka*.

यदा तु धमनीः सर्वाः कुपितोऽभ्येति मारुतः ।
तदाक्षिपत्याशु मुहुर्मुहुर्देहं मुहुश्चरं ।
मुहुर्मुहुस्तदाक्षेपक इति स्मृतः ॥
S.S.Ni/1/50-51

Ākṣepaka is characterised by vigorous movements of the body because *vāta* pervades to all the *dhamanis*. It is called *ākṣepaka* because of the repeated occurrence of the body movements (convulsions).

Pathological component

Doṣa : *Vāta pradhāna* with & without *Kapha* and *Pitta*

Dūṣya : *Rasa, rakta, sirā, dhamanī, kaṇḍarā*.

Adhiṣṭhāna : *Vātavaha nadi* and *dhamani*

Srotas : *Rasa, rakta* and *manovaha* along with *Sanjñavaha srotas*

Types of Srotoduṣṭi : *Sanḡa & atipravṛtti*

Nature of disease : Difficult to be cure

Types:

पित्तश्लेष्मान्वितो वायुर्वायुरेव च केवलः ।
कुर्यादाक्षेपकं चान्यं चतुर्थमभिघातजम् ॥
(Bh.Pr. Madh.Kh. 24/168)

1. *Vātaja*
2. *Vāta-kaphaja*
3. *Vāta-pittaja* and
4. *Abhighātaja* (due to trauma)

Clinical features: Following features are generally found in *ākṣepaka*

1. The presentation is repeatedly
2. Convulsions in the body.
3. Emaciation of limbs
4. Emaciation or deformity in veins, tendons, ligament and muscles.
5. Defective extension of muscles
6. Spasm of muscles.

अपतानक (Apatānaka)

It is caused by the factors, which is described in the context of *vātavyādhi*, but abortion, excessive haemorrhage and trauma are important etiological factors described by *Suśruta*.

Clinical features

दृष्टिं संस्तभ्य संज्ञां च हत्वा कण्ठेन कूजति ।
हृदि मुक्ते नरः स्वास्थ्यं याति मोहं वृत्ते पुनः ॥
M.Ni. 22/31

सोऽपतानकसंज्ञो यः पातयत्यन्तराऽन्तरा ।
S.S.Ni 1/52

Following features are described in the text.

1. Vision becomes steady and fixed
2. Syncope/fainting attacks
3. Wheezing sound from throat
4. Remission of fits
5. Falling of persons in between the bouts of convulsion.

Sādhya-asādhya

गर्भपातनिमित्तश्च शोणितातिस्रवाच्च यः ।
अभिघातनिमित्तश्च न सिध्यत्यपतानकः ॥
S.S.Ni. 1/59

Suśruta has mentioned that *apatānaka* which is caused by abortion, haemorrhage and due to trauma becomes *asādhyā* in nature.

दण्डाकपातनक (Daṇḍākapatānaka)

Generally it is a symptoms found in advanced stage of *Aptānaka*. Caraka said this as *daṇḍaka*, while *Bhāva miśra* has described another disease *Kubjaka* in which there is hump at the back.

Clinical features

Excessively aggravated *Vāta* causes rigidity of the hands, legs, head, back, hips and lastly entire body becomes stiff like a stick. This condition is incurable.

पाणिपादशिरःपृष्ठश्रोणीः स्तब्धान्ति मारुतः ।

दण्डवत्स्तब्धगात्रस्य दण्डकः सोऽनुपक्रमः ॥

C.S.Ci. 28/51-52

Suśruta defines that vitiated *vāta doṣa* in the process of disease, gets mixed with *āma* and *kapha doṣa* and causes the whole body stiffness like a stick.

सोऽपतानकसंज्ञो यः पातयत्यन्तराऽन्तरा ।

कफान्वितो भृशं वायुस्तास्वेव यदि तिष्ठति ॥

S.S.Ni. 1/52

Sādhyā-asādhyatā

It is *asādhyā* in nature by *Caraka* and *Vāgbhaṭa*, while *Suśruta* proclaims that it is difficult to be cure.

अन्तरायाम (Antarāyāma)

मन्ये संश्रित्य वातोऽन्तर्यदा नाड्यौ प्रपद्यते ।

मन्यास्तम्भं तदा कुर्यादन्तरायामसंज्ञितम् ॥

अन्तरायम्यते ग्रीवा मन्या च स्तब्धते भृशम् ।

दन्तानां दंशनं लाला पृष्ठायामः शिरोग्रहः ॥

जृम्भा वदनसङ्गश्चाप्यन्तरायामलक्षणम् ।

C.S.Ci. 28/43-45

Excessively aggravated *Vāta* inside *Manyā* (carotid regions) and afflicts the internal channels of this region causes spasticity of the neck which is called *Antarāyāma*. Neck bends inwards and the carotid region becomes exceedingly stiff associated

with clenching of the teeth, salivation, contraction of the back, stiffness of the head, yawning & rigidity of the face. These are the signs & symptoms of *Antarāyāma*.

बहिरायाम (Bahirāyāma)

बहिरायाम उच्यते ॥

पृष्ठमन्याश्रिता बाह्याः शोषयित्वा सिरा बली ।

वायुः कुर्यादनुस्तम्भं बहिरायामसंज्ञकम् ॥

चापवन्नाम्यमानस्य पृष्ठतो नीयते शिरः ।

उर उक्लिष्यते मन्या स्तब्धा ग्रीवाऽवमृद्यते ॥

दन्तानां दंशनं जृम्भा लालास्रावश्च वाग्रहः ।

जातवेगो निहन्त्येष वैकल्यं वा प्रयच्छति ॥

C.S.Ci. 28/45-48

Excessively aggravated *Vāta* located in the back side of the neck causes constriction of the external *Sirās* as a result of which the body bends like a bow which is called *Bahirāyāma*. While bending backwards like a bow, the head moves towards the back, the chest gets protruded, the *Manyās* become rigid, the neck gets squeezed and the teeth become clenched along with yawning, salivation and aphasia. When the attack becomes acute, it either leads to the death of the patient or manifest serious deformity in his body.

अर्दित (Ardita)

Ācārya Mādhavakara defines *ardita* as facial paralysis in terms of - "*Ardayatyanilovaktram, arditam janayatyatah*". It means the vitiated *vāta doṣa*, affects the face and produces *ardita*. Caraka considered that it is a disorder of involvement of half face or half of the body part while *Suśruta* proclaimed that it is a disorder of only involvement of half face, which is true now a days in the light of conventional medicine.

Etiological factors, signs and symptoms, pathogenesis and prognosis of *Ardita* are as follows

गर्भिणीसूतिकाबालवृद्धक्षीणेष्वसूक्ष्मेषु ।

उच्चैर्व्याहरतोऽत्यर्थं सादतः कङ्गिनानि च ॥

हसतो जृम्भतो भाराद् विषमाच्छयनादपि ।

शिरोनासौष्ठचिबुकललाटेक्षणसन्धिगः ॥

अर्दयित्वाऽनिलो वक्त्रमर्दितं जनयत्यतः ।
 वक्त्रीभवति वक्त्रार्धं ग्रीवा चाप्यपवर्तते ॥
 शिरश्चलति वाक्सङ्गो नेत्रादीनां च वैकृतम् ।
 ग्रीवाचिबुकदन्तानां तस्मिन् पार्श्वे तु वेदना ॥
 यस्याग्रजो रोमहर्षो वेपथुर्नेत्रमाबिलम् ।
 वायुरूर्ध्वं त्वचि स्वापस्तोदो मन्याहनुग्रहः ॥
 तमर्दितमिति प्राहुर्व्याधिं व्याधिविशारदाः ॥
 क्षीणस्यानिमिषाक्षस्य प्रसक्तं सक्तभाषिणः ।
 न सिध्यत्यर्दितं गाढं त्रिवर्षं वेपनस्य च ॥

S.S. Ni 1/68-73

The pregnant women, women after delivery, children, old persons, the debilitated, anemic persons, those who speak loudly, eating hard foods, laughing, yawning, weight lifting, improper postures in bed etc.

Due to above etiological factors Vāta gets aggravated and localized in places such as head, nose, lips, chin, forehead, and eyes and develops pain in these areas & cause Ardita.

Pathological component of Ardita

Doṣa : Vāta dominant Tridoṣa
Dūṣya : Rasa, rakta, māṁsa, sirā, snāyu, kaṇḍarā
Adhiṣṭhāna : half of the face
Srotas : Rasa, rakta and vātavaha srotasa.
Types of Srtoduṣṭi : Saṅga, Vimārga gamana
Status of Agni : Viṣamāgni
Nature of disease : In early stage- curable
 In chronic stage- difficult to be cure.

अर्दित लक्षण (Ardita Lakṣaṇa)

Half of the face along with neck gets crooked; trembling in head; loss of speech; abnormal appearance of eye, etc., and there is twisting of neck, chin and teeth on the affected side.

अर्दित पूर्वरूप (Ardita Pūrvarupa)

Premonitory signs and symptoms of Ardita are as follows- Horripilations, tremors, turbidity of the eyes, upward movement of Vāta, numbness in skin, pricking pain, and stiffness in the sides of the neck & jaws.

साध्यासाध्यता (Sādhyāsādhātā)

Ardita is incurable in persons who are emaciated, having fixed eyes and constantly muffled voice, which has persisted for than three years associated with shaking of the head.

पक्षाघात एवमर्दित (Pakṣāghāta Evam Ardita)

अतिवृद्धः शरीरार्धमेकं वायुः प्रपद्यते ।
 यदा तदोपशोष्यामृग्याहुं पादं च जानु च ॥
 तस्मिन् सङ्कोचयत्यर्थं मुखं जिह्वं करोति च ।
 वक्त्रीकरोति नासाभूललाटाक्षिह्नूस्तथा ॥
 ततो वक्त्रं व्रजत्यास्ये भोजनं वक्त्रदर्शिनः ।
 स्तब्धं नेत्रं कथयतः क्षवयुश्च निगृह्यते ॥
 दीना जिह्वा समुत्क्षिप्ता कला सज्जतिचास्य वाक् ।
 दन्ताश्चलन्ति बाध्येते श्रवणौ भिद्यते स्वरः ॥
 पादहस्ताक्षिजङ्घोरुशङ्खश्रवणगण्डरुक् ।
 अर्थे तस्मिन्मुखार्धे वा केवले स्यात्तदर्दितम् ॥

C.S.Ci. 28/38-42

Due to consumption of Vāta aggravating etiological factors, Vāta becomes excessively excacerbated leading to afflictions half of the body and dries up the blood, hand, foot and knee and develops contracture in these parts. It causes distortion in half of the face, and curvature of the nose, eyebrows, forehead, eye and mandible. Ingested food moves tortuously to the one side of the mouth or remnants of the food; while speaking, the nose becomes curved & the eyes remains fixed; sneezing is suppressed inspite of impulse; tongue when raised becomes curved; voice becomes feeble and impeded; looseness of teeth; hearing is deficient; hoarseness of voice; pain in foot, hand, eye, calf, thigh, temple, ear & cheek. The disease may confined only to half of the body or in half of his face is known as Ardita.

Types of Ardita: Its types are not clearly mentioned in the classical texts of Ayurveda, but in Bhāvaprakāśa three types of ardita has been described such as- (Bh.Pr. madh.Kh. 24/64)

1. Vāta predominant
2. Pitta predominant
3. Kapha predominant

Clinical presentation

- Vitiated *vāta doṣa* produces contracture in half of the body or half of the face or both along with face and nose, eye brows, forehead, eyes and jaws also get crooked.
- There is excessive salivation and the eye on the affected side remains partially closed. Thus when the food is taken, the morsel goes on the affected side and the tongue also get affected.
- Patient may partially or completely lose his voice and there is pain in foot, hand, eyes, thigh, temple, ear and cheek.
- Besides, features like-involuntary movement of head, aphonia, immobilization of eyes, frictions of teeth, hoarseness of voice, deafness, difficulty in sneezing, anosmia, altered sensorium, fear at the time of sleep etc are described by *Vāgbhaṭa*. (C.S.Ci. 28/38-42, S.S.Ni. 1/71, A.H.Ni. 15/34-36)

Features of *Ardita* according to predominance of *Doṣas*:

1. *Vāta doṣa*: this is accompanied with excessive salivation, tremors, swelling and pain in affected lips and limbs.
2. *Pitta doṣa*: this is accompanied with fever, fainting and excessive thirst.
3. *Kapha doṣa*: there is oedema and stiffness of cheeks, neck and head. (Bh.PR. Madh.Kh. 24/65-66)

पक्षाघात (Pakṣāghāta)

Nidāna (aetiology)

Etiological factors of the disease are as follows-

1. All the etiological factors mentioned for *Vātavyādhi*
2. Bloodletting therapy

If ignorant physician perform bloodletting therapy in too hot season & after excessive sudation therapy as a result blood flows out excessively resulting in the manifestation of *Pakṣāghāta*.

अत्युष्णोऽतिस्वित्रोऽतिविद्धोऽज्ञैर्विस्त्रावितमतिप्रवर्तते ।
तदतिप्रवृत्तं शिरोऽभितापमान्धमधिमन्थतिमिरप्रादुर्भावं

धातुक्षयमाक्षेपकं पक्षाघातमेकाविकारं । S.S.Su. 14/30

Pakṣāghāta may manifest in pregnant women, women after delivery, old people, emaciated persons & that caused by heavy bleeding and loss of sensation in the affected limbs should be refused for treatment.

गर्भिणी मृतिकाबालवृद्धक्षीणेऽप्यमुक्नुते ।
पक्षाघातं परिहरेद् वेदनारहितो यदि ॥

M.Ni. 22/43

Pakṣāghāta manifest due to injury to *Lohita marma* & *Kakṣadhara marma*.

तत्र लोहितक्षयेण मरणं पक्षाघातो वा ।

S.S.Śā. 6/25

कक्षधरे पक्षाघातः । S.S.Śā. 6/25

Samprāpti & Lakṣaṇa

1. Aggravated *Vāta* seizes power of half of the body i.e. on either right or left cause's dryness of *Sirā* & *Snāyu* and manifest symptoms like loss of movement, pain and loss of speech. *Vāta* causes contracture in one of the feet or hands with piercing pain & distress, is the characteristic feature of *Ekāṅga roga*. If the affliction of *Vāta* is generalised, this is known as *Sarvāṅga roga*.

2. Excessively aggravated *Vāta* inside the *Dhamani's* moves downwards, obliquely and upwards and paralyse on either side of the body i.e. right or left by loosening the bonds of the bindings of the joints is called *Pakṣāghāta* & it is characterised by loss of functions in half portions of the body, unconsciousness, falls down or dies because of being afflicted with *Vāta*.

If *Pakṣāghāta* manifest by *Vāta* alone is regarded as curable with great difficulty; if it is associated with either *Pitta* or *Kapha Doṣa* then it becomes curable and while that caused by depletion of *Dhātus* is incurable.

हत्वैकं मारुतः पक्षं दक्षिणं वाममेव वा ॥

कुर्याच्चेष्टानिवृत्तिं हि रुजं वाक्स्तम्भमेव च ।

गृहीत्वाऽर्धं शरीरस्य सिराः स्रायूर्विशोष्य च ॥

पादं संकोचयत्येकं हस्तं वा तोदशूलकृत् ।

एकाङ्गरोगं तं विद्यात् सर्वाङ्गं सर्वदेहजम् ॥

C.S.Ci. 28/53-55

पक्षाघातं भिषग्वराः ॥

यस्य कृत्स्नं शरीरार्धमकर्मण्यमचेतनम् ।
ततः पतत्यसन् वाऽपि जहात्यनिलपीडितः ॥
शुवातहतं पक्षं कृच्छ्रसाध्यतमं विदुः ।
साध्यमन्येन संसृष्टमसाध्यं क्षयहेतुकम् ॥

S.S.Ni. 1/61-63

If *Pakṣāghāta* manifest due to association with *Pitta* it manifests burning sensation, increased body temperature and fainting along with other classical features of *Pakṣāghāta*. If *Pakṣāghāta* manifest due to association with *Kapha*, it manifests clodness, swelling, heaviness along with other classical features of *Pakṣāghāta*.

दाहसन्तापमूर्च्छाः स्युर्वायौ पित्तसमन्विते ।
शैत्यशोथगुरुत्वानि तस्मिन्नेव कफान्विते ॥

M.Ni. 22/42-43

Types of *Pakṣāghāta*: Depending upon the body part involved, it can be divided into following three types:

1. *Ekāṅga ghāta*: Monoplegia- in this state only one hand or leg get affected.
2. *Adharāṅga ghāta*: Paraplegia- in this state both the lower extremities are affected.
3. *Sarvāṅga ghāta*: Diplegia- most of the times this is congenital defect and both arms and legs get paralyzed.

Sādhyaśādhya

1. *Pakṣāghāta* which has been developed only due to the vitiation of *Vāta doṣa* is *kaṣṭasādhya*.
2. If it is develop due to aggravation of *Vāta* due to loss of tissues, then it is also *kaṣṭasādhya*.
3. The one that is develop due to vitiation of *kapha & pitta* along with *Vāta* is *Sādhya*.
4. If it affects pregnant women, children, old people and debilitated persons then it is incurable.
5. If it is painless type then it is also incurable.
(S.Ni. 1/63) (Bh.Pr. Madh.Kh 24/208)

In conventional system of medicine, paralysis have strong correlation with *Pakṣāghāt* of Ayurveda. It is loss of the ability to move one or more muscles. It may be associated with loss of

feeling and other bodily functions. Paralysis is not usually caused by problems with the muscles themselves, but by problems with the nerves or spinal cord that the brain uses to control muscles. Therefore, a person with paralysis will usually have some form of nerve damage

कम्पवात-*Kampavāta* (Parkinsonisms)

Kampavāta is one of the *Vāta nānātma* *vyādhi* as explained by Caraka in *Sūtrasthāna* chapter 20. It is also named as *Vepathu* by various other scholars of Ayurveda, which have similar meaning to *Kampavāta*. Features of Parkinson's disease of conventional system of medicine are comparable to the *Kampavāta* of Ayurveda. Parkinson's disease, it is also known as idiopathic or primary parkinsonism, hypo-kinetic rigid syndrome, or paralysis agitans. It is a degenerative disorder of the central nervous system. The motor symptoms of Parkinson's disease result from the death of dopamine-generating cells in the substantia nigra, a region of the midbrain; the cause of this cell death is unknown. Early in the course of the disease, the most obvious symptoms are movement-related; later, thinking and behavioral problems may arise. Other symptoms include sensory, sleep and emotional problems. The main motor symptoms are collectively called as *Parkinsonism*. Parkinson's disease is more common in older people, with most cases occurring after the age of 50.

*Karapādatale kampo Dehabhramāṇadukhite I
Nidrābhango matiḥ kṣiṇā Kampavātsya
lakṣaṇam II*

(*Basavarājīyama*)

1. Tremors in hands and feet
2. Irregular movement of body parts
3. Insomnias and
4. Reduced memory and intelligence

Conventional features of Parkinson's disease

1) Motor symptoms

- Bradykinesia- difficulty in initiating movements and slowness during executing

movements

- Difficulty in getting from chair
- Hypophonia
- Tremor-Pill rolling type
- Rigidity
- Cog wheeling sensation
- Dystonia- muscle dysfunction characterized by spasms or abnormal muscle contraction.

2) Non-motor symptoms

- Depression & anxiety
- Cognitive impairment
- Sleep disturbance
- Sensory abnormality & Pain
- Loss of Smell
- Incontinence of urine
- Constipation

3) Neuropsychiatric symptoms

- Change in mood
- Altered cognition and behavior

गृध्रसी (Grdhrasī)

Excessively aggravated *Vāta* inside buttocks causes rigidity, pain and pinning sensation and gradually descends to low-back, knees, calves and legs. There will be repeated pulsation in these areas. *Vātakaphaja Grdhrasī* is characterised by drowsiness, heaviness and anorexia.

स्फिक्पूर्वा कटिपृष्ठोरुजानुजङ्घापदं क्रमात् ।
गृध्रसी स्तम्भरुक्तोर्दग्ध्वाति स्पन्दते मुहुः ॥
वाताद्वातकफात्तन्द्रागौरवारोचकान्विता ।

C.S.Ci. 28/56-57

Excessively aggravated *Vāta* inside buttocks causes rigidity, pain and pinning sensation and gradually descends to low-back, knees, calves and legs. There will be repeated pulsation in these areas.

Vātakaphaja Grdhrasī is characterised by drowsiness, heaviness and anorexia.

पार्णिप्रत्यङ्गुलीनां तु कण्डरा याऽनिलादिता ।
सक्थः क्षेपं निगृहीयाद्गृध्रसीति हि सा स्मृता ॥

S.S.Ni. 1/74

Excessively aggravated *Vāta* enters the *Kaṇḍarās* of the ankles & toes causes *Kṣepa* (pain & inability to extend) in the thighs, is known as *Grdharasī*.

विश्वाची गृध्रसी चोक्ता खल्ली तीव्ररुजान्विते ।

A.H.Ni. 15/55

Viśvācī and *Grdhrasī* when accompanied with severe pain are called *Khallī*

Pathological component

Doṣa : *Vāta* or *Vāta-kapha*

Dūṣya : *Rasa, rakta, māṁsa, meda, snāyu, kaṇḍarā* and *sirā*

Adhiṣṭhāna : *Kaṭipradeśa* and radiated towards posterior aspect of lower limb.

Srotas : *Rasa, rakta, mamsa, meda, asthi, vāta* and *sanjnavaha srotas*

Types of srotoduṣṭi : *Atipravṛtti, saṅga* and *sirāgrānṭhi*.

Nature of disease : Acute or chronic

विश्वाची (Viśvācī)

The *Viśvācī* of Ayurveda is comparable to the brachial neuralgia or brachial neuritis of modern medicine, which is a radiating pain in the shoulder and upper arm caused by damage to a nerve in the brachial plexus of neck. It is caused by the structural nerve compression in the neck region, or it may be caused by the damage of nerves as a result of infection, inflammation, or progressive neurological disease. The pain may be sharp and stabbing and can come and go, depending on the cause. Many patients find it severe and intolerable, and usually seek treatment quickly.

Ayurveda described that excessively aggravated *Vāta* enters the *Kaṇḍarā* of the palm, fingers and back of arms causes impairment or loss of sensory and motor functions of the same. It is known as *Viśvācī*.

तलं प्रत्यङ्गुलीनां तु कण्डरा बाहुपृष्ठतः ।
बाहोः कर्मक्षयकरी विश्वाचीति हि सा स्मृता ॥

S.S.Ni. 1/75

Clinical features

Following are the signs and symptoms of brachial neuritis/neuralgia.

- Sudden onset of pain in shoulder, arm and neck or hand
- Weakness in arm and shoulder
- Difficulty moving arm and shoulder
- Pins and needles
- Numbness in arm
- Shoulder and hand
- Fatigue
- Malaise
- Headache
- Muscle aches and pains

Diagnosis and test

- NCV
- X-ray AP & lateral view

- MRI of cervical spine to rule out the cause of compression

खञ्ज-पङ्गु (Khañja-Paṅgu)

Pathogenesis and clinical features

वायुः कट्यां स्थितः सक्थः कण्डरामाक्षिपेद्यदा ।
तदा खञ्जो भवेज्जन्तुः पङ्गुः सक्थोर्द्वयोरपि ॥

A.H.Ni. 15/45

Excessively aggravated *Vāta* in the waist invades the big tendons of the legs and then the person becomes *Khañja* (lame by one leg) and *Paṅgu*, when both legs are affected.

वायुः कट्यां स्थितः सक्थः कण्डरामाक्षिपेद्यदा ।
खञ्जस्तदा भवेज्जन्तुः पङ्गुः सक्थोर्द्वयोर्वधात् ॥

S.S.Ni. 1/77

Vāta gets vitiated by its own etiological factors and affects one of the important *kandara* in the leg. This causes very severe pain and constriction in the muscle. Hence the affected is unable to move properly and starts limping. This condition is called as *khanja*. When both legs are affected the person become paralysed and is called as *pangu*.

Differentiate between *Gr̥dhrasī* and *Viśwācī*

S.No.	<i>Gr̥dhrasī</i>	<i>Viśwācī</i>
Clinical features	Excessively aggravated <i>Vāta</i> <i>Kandaras</i> of the ankles & toes causes <i>Kṣepa</i> (pain & inability to extend) in the thighs, is known as <i>Gr̥dhrasī</i> . Excessively aggravated <i>Vāta</i> inside buttocks causes rigidity, pain and pinning sensation and gradually descends to low-back, knees, calves and legs. There will be repeated pulsation in these areas.	y aggravated <i>Vāta</i> enters the <i>Kandara</i> of the palm, fingers and back of arms causes loss of functions in arms. It is known as <i>Viśwācī</i> .
Classification	1. <i>Vātaja Gr̥dhrasī</i> 2. <i>Vātakaphaja Gr̥dhrasī</i> is characterised by drowsiness, heaviness and anorexia.	No classification
Contemporary correlation	Sciatica	Radioulnar paralysis

Differentiate between Grdhrasī and Khañja

S.No.	Grdhrasī	Khañja
Clinical features	Excessively aggravated Vata enters the <i>Kandaras</i> of the ankles & toes causes <i>Kṣepa</i> (pain & inability to extend) in the thighs, is known as <i>Grdhrasī</i> . Excessively aggravated Vata inside buttocks causes rigidity, pain and pinning sensation and gradually descends to low-back, knees, calves and legs. There will be repeated pulsation in these areas.	Excessively aggravated vata in the waist invades either in the right or left big tendons of the legs and due to destruction in either of the thighs causes <i>Khañja</i> and makes him limp
Classification	1. <i>Vātaja Grdhrasī</i> 2. <i>Vātakaphaja Grdhrasī</i> is characterised by drowsiness, heaviness and anorexia.	No classification
Contemporary correlation	Sciatica	Monoplegia

Differentiate between Kalayakhañja and Khañja

S.No.	Kalayakhañja	Khañja
Clinical features	<i>Kalayakhañja</i> is characterised by shaking of the leg at the commencement of walking and limps while walking and binding of the joints will be loosened.	Excessively aggravated vata in the waist invades either in the right or left big tendons of the legs and due to destruction in either of the thighs causes <i>Khañja</i> and makes him limp
Contemporary correlation	Lathyrism	Monoplegia/Limp

MCQs

- a. व्यपेताहिका (Vyapeta Hikka)
 - a. *Apatantraka*
 - b. *Antaraayama*
 - c. *Bahyaayama*
 - d. *Ardita*
2. *Dhanushkampa* is
 - a. *Apatantraka*
 - b. *Antaraayama*
 - c. *Bahyaayama*
 - d. *Ardita*
3. *Kapothaivakujathe* is the feature of
 - a. *Apatantraka*
 - b. *Antaraayama*
 - c. *Kshatajakasa*
 - d. *Kshayaajakasa*
4. Vata get vitiated in _____ in *Akshepaka*
 - a. *Kandara*
 - b. *Sira*
 - c. *Snayu*
 - d. *Dhamani*
5. *Jihwaathilekhana* is the *Nidana* of
 - a. *Jihwasthambha*
 - b. *Pakshaghata*
 - c. *Hanusramsa*
 - d. *Ardita*
6. Prognosis of *Sudhavatagata Pakshaghata*
 - a. *Sukhasadhya*
 - b. *Krchrasadhya*
 - c. *Yapya*
 - d. *Krichrasadhyatama*
7. *Bahucheshatapaharini* is
 - a. *Visvachi*
 - b. *Apabahuka*
 - c. *Pangu*
 - d. *Khanja*
8. Types of *Grdhrasi* as per *Caraka*
 - a. 3
 - b. 1
 - c. 2
 - d. 5

9. 'Ekaanga-Roga' is

- a. Ardita
c. Khanja

- b. Pakshaghata
d. Pangu

10. When disturbed Vayu enters into Snayu causes

- a. Gridrasi
c. Apatanaka

- b. Vishwachi
d. Aksepaka

11. Kricchrath charvana bhashanam is the Nidana of

- a. Jihwasthambha
c. Hanusramsra

- b. Pakshaghata
d. Ardita

12. Site of Samprapti of Jihwasthambha

- a. Hanumoolasira
c. Vakvahasira

- b. Dhamani
d. Jathrurdhwasira

Answer

1. d	2. b	3. a	4. d	5. c	6. d
7. a	8. c	9. b	10. d	11. c	12. c